

Of course. Here is a highly realistic discharge summary formatted to look like a raw text extraction from a structured hospital document.

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KK WOMEN'S AND CHILDREN'S HOSPITAL

100 Bukit Timah Road
Singapore 229899
Tel: (+65) 6225 5554
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HOSPITAL DISCHARGE SUMMARY

--- PATIENT INFORMATION

PATIENT NAME : SUBRAMANIAM, PRIYA
NRIC/FIN : S****5678A
DATE OF BIRTH : 1993-08-22 (31 Years)
GENDER : Female
ADDRESS : 15 Marine Parade Rd, #12-04, Singapore 449267
CONTACT : +65 9123 4567

--- ADMISSION DETAILS

ADMISSION DATE : 2025-05-12 19:45
DISCHARGE DATE : 2025-05-16 11:30
HOSPITAL CASE NO. : KKH-25-0089142
WARD / BED : 6A / 21B (Single Room)
ATTENDING PHYSICIAN : Dr. Eileen Tan (MCR: 12345J)
Senior Consultant, Obstetrics & Gynaecology

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PRINCIPAL DIAGNOSIS:
082.9 - Delivery by Caesarean section, unspecified.
- Term pregnancy at 39+4 weeks, delivered.

SECONDARY DIAGNOSES:
066.9 - Obstructed labor, unspecified (Failure to progress in first stage).
Z37.0 - Singleton, liveborn.

PROCEDURES PERFORMED

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	PROCEDURE	DATE SURGEON
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| Lower Segment Caesarean Section (LSCS) | 2025-05-13 | Dr. Eileen Tan |
| Spinal Anaesthesia | 2025-05-13 | Dr. Lim Wei Ming |

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SUMMARY OF HOSPITAL COURSE:

Patient, a 31-year-old G1P0 lady at 39+4 weeks gestation, was admitted on 2025-05-12 for management of labor. She presented with regular uterine contractions. On examination, cervix was 3cm dilated. Admitted to the delivery suite for monitoring and augmentation of labor.

Labor was augmented with an oxytocin infusion. Despite adequate contractions over the next 8 hours, cervical dilatation progressed poorly from 4cm to 5cm. Fetal heart rate monitoring remained reassuring throughout. The lack of progress was assessed as cephalopelvic disproportion / failure to progress.

After counselling the patient and her husband on the findings and options, a joint decision was made to proceed with a Lower Segment Caesarean Section (LSCS).

Patient underwent an uncomplicated LSCS under spinal anaesthesia on 2025-05-13 at 08:52. A live, healthy baby boy was delivered.

- Baby's Weight: 3.45 kg
- APGAR Scores: 9 at 1 minute, 10 at 5 minutes.
- No neonatal complications noted. Baby rooming-in with mother.

Post-operatively, the patient's recovery was uneventful. Vital signs remained stable. Pain was well-controlled with oral analgesia. The surgical wound is clean, dry, and intact. Patient was able to ambulate independently from post-op Day 2. Lochia is normal. Breastfeeding was successfully initiated.

CONDITION ON DISCHARGE:

Stable, afebrile, and comfortable. Ambulating well. Tolerating diet. Passing flatus and urine normally. Post-operative wound is satisfactory.

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DISCHARGE MEDICATIONS:

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MEDICATION	DOSAGE	FREQUENCY	DURATION
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Tab Paracetamol	1000 mg	Every 6 hours, PRN	7 days
Tab Ibuprofen	400 mg	Every 8 hours, PRN	7 days
Ferrous Fumarate	200 mg	Once daily	30 days

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FOLLOW-UP INSTRUCTIONS:

1. Appointment at Women's Specialist Clinic C with Dr. Eileen Tan for postnatal review and wound check in 6 weeks. Date: 2025-06-25 at 10:00 AM.
2. Remove waterproof dressing over wound after 7 days. Keep wound clean and dry.
3. Watch for signs of wound infection (redness, swelling, discharge, fever).
4. Continue postnatal and lactation support as advised.

5. Newborn to follow up at local Polyclinic for developmental checks and vaccinations.

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FINANCIAL SUMMARY FOR INSURANCE CLAIM

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ITEM DESCRIPTION	AMOUNT (SGD)
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ROOM CHARGES (Single Room, 3 Nights)	1,860.00
DELIVERY ROOM / OPERATING THEATRE CHARGES	1,950.00
OBSTETRICIAN'S PROCEDURE FEE (LSCS)	2,800.00
ANAESTHETIST'S FEE	950.00
NURSERY & NEONATAL CARE	420.00
MEDICATIONS & CONSUMABLES	145.00
LABORATORY & DIAGNOSTIC TESTS	75.00
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TOTAL BILL AMOUNT	8,200.00
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Electronically Signed,

Dr. Eileen Tan
Senior Consultant
Department of Obstetrics & Gynaecology
KK Women's and Children's Hospital
MCR No: 12345J